

Five demands for people with dysmelia

Dysmelia is a congenital limb difference. DysNet is the network of the associations that represent the families concerned, and it carries these five demands into every body where it holds a seat: EURORDIS, the European Disability Forum, ERN BOND and the European Economic and Social Committee. Each demand states what we ask of public authorities, and what would count as progress.

1 Recognition of dysmelia as a public health priority, and access to quality healthcare for every family affected

Name dysmelia in national rare-disease and disability plans, and guarantee every child and adult a pathway to a competent team: diagnosis, surgery when useful, prosthetics, rehabilitation and psychological support, wherever the family lives.

PROGRESS LOOKS LIKE national pathways published, reference centres named, and waiting times measured.

2 Universal coverage of prosthetics, research that reaches people with dysmelia, and an orphan medical devices framework that makes equipment affordable

Cover a functional prosthesis for everyone who wants one, renewed at the pace of a growing child; fund research that names congenital limb difference; and write an orphan medical devices status into law, with fee relief and priority assessment.

PROGRESS LOOKS LIKE comparable reimbursement rules across countries, research calls that name our conditions, a legal definition and public register of orphan devices, and families' remaining costs measured and falling.

3 Registries that cover whole populations, interoperability and portability of their data, and personal data returned to the people concerned

Fund registries that cover whole populations, make them interoperable and portable across borders, and return to each person a copy of their own record, with consent they can see and change.

PROGRESS LOOKS LIKE coverage figures rising, cluster investigations that can compare notes internationally, common data models adopted, published access procedures, consent that families can see and change, and records that families can download and carry with them.

4 Research that looks for the causes of dysmelia, not only for how often it happens

Fund research whose object is causation, not only frequency: exposure histories linked to registry records, standing protocols for investigating clusters, toxicology on the substances already suspected, and the publication of negative results.

PROGRESS LOOKS LIKE calls for proposals that name the causes of congenital limb anomalies as their subject, and a causal literature that grows faster than the count of cases.

5 Precaution first: science-based information and enforceable rules on products with suspected, potential or proven teratogenic effects

Apply the precautionary principle to substances with suspected, potential or proven teratogenic effects. The list is needed at two levels: one WHO reference list, so a substance is recognisable across borders, and a register in each state with legal force, publishing the evidence and the reasoning behind every approval and every prohibition. With it, clear information to families and professionals, and enforceable obligations on food suppliers, the construction sector and product manufacturers.

PROGRESS LOOKS LIKE one WHO reference list of substances of concern, a national register in each state with legal force and published reasons, mandatory labelling and disclosure, and inspections with consequences.

What you can do

Name one of these five demands in your next meeting, consultation response or national plan, and tell us which: we will send the evidence behind it.

The registers that support these demands are public and free to cite — teratogens, care centres, population registries and our bibliography, at www.dysnet.org/knowledge/. Write to info@dysnet.org.